

ALCOHOL AND/OR DRUG USE

Unhealthy alcohol use refers to a pattern of use that puts the patient at risk of dependence and physical, mental and social harm. Any drug use is unhealthy. If patient smokes, encourage to stop ↗ 141.

Assess the patient with unhealthy alcohol use or any drug use

Assess	Note
Symptoms	- If recently reduced/stopped use and restless, agitated, difficulty sleeping, confused, anxious, hallucinating, sweating, tremors, headache or nausea/vomiting, treat for likely withdrawal ↗ 85. - If aggressive/violent or disruptive behaviour ↗ 84. - If patient has suicidal thoughts or plans, refer same day ↗ 83.
Harmful use	- Assess quantity and frequency of alcohol use: if drinking > 14 drinks¹/week or ≥ 4 drinks¹/session, explain that this increases risks of harm and dependence. - Look for harm: physical harm (like injuries, liver disease, stomach ulcer), mental harm (like depression), social harm (relationship, legal or financial) or risky behaviour.
Dependence	Patient is dependent if ≥ 3 of: strong need to use substance; difficulty controlling use; withdrawal on stopping/reducing; tolerance (needing more); neglecting other interests; continued use despite harm.
Stressors	Help identify domestic, social and work factors contributing to alcohol/drug use. Ask about reasons for his/her substance use. If patient is being abused ↗ 88.
Mental health	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↗ 143. If stress or anxiety ↗ 86.

Advise the patient with unhealthy alcohol use or any drug use

Health for All

↗ 37 and ↗ 41

- If pregnant/planning pregnancy or breastfeeding, advise to avoid alcohol/drugs completely. Alcohol/drugs can harm the developing baby.
- Suggest patient seeks support from close relatives/friends who do not use alcohol/drugs, a support group or a helpline ↗ 178. Refer patient to social worker, psychologist or counsellor.
- Discuss risks/harms that using alcohol/drugs may cause. Allow patient to decide for him/herself to stop or cut down. Support the patient to make a change ↗ 177.

Unhealthy alcohol use without dependence

- If pregnant, harmful drinking, previous dependence problem or contraindication (like liver damage, mental illness), advise to stop alcohol completely. Avoid drinking places and keeping alcohol at home.
- If none of above and patient chooses to continue alcohol, advise to at least cut down to low-risk alcohol use: ≤ 2 drinks¹/day and avoid alcohol on at least 2 days of the week.

Any drug use without dependence

- Advise to stop using illegal or misusing prescription drugs completely.
- If patient chooses to continue, advise to reduce harm: avoid injections or use sterile injection technique, test regularly for HIV and hepatitis.
- Consider the need for PrEP ↗ 106.

Alcohol/drug dependence

Advise that alcohol/drugs need to be stopped slowly. If alcohol/drugs stopped suddenly, withdrawal effects can be harmful. Detoxification (below) will safely wean the body from alcohol or drug/s.

If alcohol/drug dependence, doctor to treat the patient with the help of the carer

- Arrange inpatient detoxification if previous withdrawal delirium/fits or failed detoxification, pregnant, chronic medical or mental illness, homeless/no social support, dependent on opioid or > 1 drug.
- Doctor can do outpatient detoxification if none of the above. Ensure patient has a close relative/friend to act as supervisor during programme.

Substance	Detoxification programme - Write out programme for patient and chosen supervisor
Alcohol	- Give thiamine 300mg daily for 14 days. - Give diazepam 10mg with withdrawal symptoms then 5mg 6 hourly for 3 days. Then 5mg 12 hourly for 2 days. Then 5mg daily for 2 days. Then stop. If withdrawal symptoms persist despite this, refer/discuss.
Cannabis/Tik/ Cocaine/Mandrax	- Medication is not always needed. - Treat anxiety or sleep problems with diazepam 5mg daily or 12 hourly, tapering over 5-7 days. Monitor for depression and psychosis.
Benzodiazepines	- Avoid suddenly stopping benzodiazepines. Withdrawal may take months. - Replace benzodiazepine patient is taking with diazepam. If taking lorazepam 0.5mg-1mg, replace with diazepam 5mg. For other benzodiazepines, refer to SAMF, MIC hotline or substance helpline ↗ 178. - Decrease diazepam every 2 weeks by 2-2.5mg. If symptoms occur, continue or increase dose for 2 more weeks. Once at 20% of initial dose, decrease by 0.5-2mg every week.

Review the patient on a detoxification programme daily until stable. Advise to return immediately if any problems. Stop programme if patient resumes alcohol/drug use.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.